ED NOTE

Patient John Doe	**DOB / Age / Gender** 05/12/1974 / 52Y / Male	**Account** #99841201
Date of Service 10/04/2026	**Attending** Dr. R. Vance, MD	**Encounter** Emergency Department

ED Presentation

52-year-old male with hypertension and hyperlipidemia presented to the Emergency Department with sudden-onset substernal crushing chest pain for approximately 2 hours, rated 9/10, radiating to the left arm and jaw, with diaphoresis, dyspnea, and mild nausea. ECG showed 3 mm ST elevation in II, III, and aVF with reciprocal ST depression in I and aVL. High-sensitivity Troponin I was 4.85 ng/mL. The record describes an acute inferior-wall STEMI and urgent cardiac catheterization/PCI disposition.

Vital Signs

BP	HR	RR	Temp	SpO2
168/98	112 bpm	24/min	98.4 °F	92% RA → 97% on 2L O■ NC

Physical Examination

Severe distress with diaphoresis and anxiety. Tachycardic regular rhythm, normal S1/S2, no S3/S4/rub. Mild bibasilar crackles. No pedal edema. Alert and oriented x4.

ED Interventions

Medication	Documented administration
Aspirin	325 mg chewed at 14:15
Nitroglycerin	0.4 mg SL × 3; partial relief
Heparin	4,000 units IV bolus
Clopidogrel	600 mg oral loading dose
Morphine	2 mg IV

ED Disposition

Urgent cardiac catheterization/CCU admission for emergency PCI.

Source note: Generated only from the supplied 4-page training medical record. Where a requested note type was not explicitly documented, no unsupported clinical details were added.